

ROYAL FITNESS CLUB

RFC PREMIUM GUIDE #16

BEGINNER ANABOLIC CYCLE COMPLETE GUIDE

Your First Cycle: Protocol, Safety, PCT & Long-Term Strategy

12 Weeks · 6 Chapters · Beginner Level · Full Safety Protocol

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DISCLAIMER

For educational and harm-reduction purposes only. Not medical advice. Consult a qualified healthcare professional before starting any protocol. Individual results vary.

CHAPTER 1 — READINESS ASSESSMENT

"The beginner who enters their first cycle with complete information and a clear protocol is safer than the advanced user who has become complacent."

A beginner anabolic cycle requires careful preparation. The foundation of safe cycle use is pre-cycle blood work, a clear compound selection strategy, an on-cycle support protocol, and a confirmed PCT plan before the cycle begins. Never start a cycle without PCT drugs already secured and ready.

Pre-Cycle Checklist	Status Required	Why Critical
Baseline blood work completed	Full panel done	Cannot identify suppression without baseline
Training age	2+ years structured training	Genetic ceiling should be near-reached first
Age	21+ (25+ recommended)	HPTA still maturing before 21
PCT drugs secured	Clomid + Nolvadex in hand	Never start without PCT available
Support supplements	Liv52, omega-3, BP monitor	Organ protection starts on Day 1
Trusted contact aware	Someone knows your cycle	Emergency preparedness

CHAPTER 2 — TESTOSTERONE-ONLY PROTOCOL

The gold standard first cycle is Testosterone Enanthate 400–500mg per week for 12 weeks. This single-compound protocol maximises gains while making side-effect management straightforward — if anything goes wrong, there is only one compound to adjust.

Week	Test E (Mon)	Test E (Thu)	Weekly Total	Key Action
1–2	200mg	200mg	400mg	Assess injection tolerance, no side effects expected
3–4	250mg	250mg	500mg	Full dose — monitor for estrogen symptoms
5–10	250mg	250mg	500mg	Consistent dose — bloodwork at Week 6
11–12	250mg	250mg	500mg	Final push — track progress photos
13–14	None	None	0mg	Clearance period before PCT
15–18	PCT only	—	—	Clomid 50/50/25/25 + Nolvadex 40/40/20/20

■ ***Use only Testosterone Enanthate or Cypionate for your first cycle. Propionate requires more frequent injections and is not ideal for beginners.***

CHAPTER 3 — ON-CYCLE ESSENTIALS

Support Item	Dose	Purpose	Start
Liv52 DS / TUDCA	2 tabs Liv52 or 500mg TUDCA	Liver protection	Day 1
Omega-3 fish oil	4g EPA/DHA combined	Lipid protection, cardiovascular	Day 1
CoQ10	200mg/day	Heart and mitochondrial support	Day 1
Vitamin D3	5000 IU/day	Hormonal co-factor	Day 1
Zinc	25mg/day	Testosterone support co-factor	Day 1
Blood pressure monitor	Check weekly	Target < 130/80 mmHg	Before Day 1
Arimidex (on hand)	0.25mg 2× weekly if needed	Estrogen management IF symptoms appear	Only if needed

- Week 6 blood panel: testosterone total + free, estradiol, LH, FSH, CBC, CMP, lipids
- Estrogen symptoms: tender or itchy nipples, unusual water retention — introduce AI at low dose
- Blood pressure above 140/90: reduce sodium, increase cardio, consider reducing dose

CHAPTER 4 — TRAINING PROGRAMME

On your first cycle, increase training volume by 20–30% above your natural training. Your recovery capacity and protein synthesis are elevated — but not unlimited. Progressive overload on all main compound lifts.

Day	Muscles	Sets	Rep Range	Key Exercises
Monday	Chest + Triceps	16	8–12	Bench, incline dB, dips, cable push
Tuesday	Back + Biceps	16	8–12	Pull-ups, barbell row, cable row, curls
Wednesday	Legs	18	8–15	Squat, leg press, RDL, leg curl
Thursday	Shoulders + Traps	14	10–15	OHP, lateral raises, face pull, shrugs
Friday	Arms + Core	14	10–15	Close grip bench, hammer curls, planks
Saturday	Cardio + weak points	10–12	—	20 min LISS + 2 weak point exercises
Sunday	Rest + mobility	—	—	Foam roll, light stretching

CHAPTER 5 — POST CYCLE THERAPY

"PCT is not optional. It is the part of the cycle that determines whether this was a temporary loan or a permanent acquisition."

PCT begins 14–18 days after the last Testosterone Enanthate injection. This allows the long ester to clear the system. Starting PCT too early (with residual testosterone still suppressing the HPTA) wastes your PCT drugs.

PCT Week	Clomiphene (Clomid)	Tamoxifen (Nolvadex)	Expected Status
Week 1	50mg/day	40mg/day	Low energy, low libido — normal
Week 2	50mg/day	40mg/day	Mood swings peak — stay consistent
Week 3	25mg/day	20mg/day	Energy begins returning
Week 4	25mg/day	20mg/day	Libido should be recovering

- 4 weeks after PCT completion: blood work. Target testosterone > 80% of pre-cycle baseline
- If testosterone below 300 ng/dL at 8 weeks post-PCT: endocrinologist consultation
- Continue training hard and eating protein through PCT — hormones + training = muscle retention

CHAPTER 6 — LIFE AFTER YOUR FIRST CYCLE

Consolidating Your Gains

- Expect to retain 70–80% of cycle gains after full recovery if PCT and training are maintained
- Minimum 12 weeks between cycles (ideally cycle length = time off)
- Use the off-cycle period to maximise natural gains, solidify new techniques, run blood work
- Avoid second cycle until testosterone is confirmed recovered to at least 80% of pre-cycle baseline
- Each subsequent cycle should have a clear, specific goal — not just "to do another one"

Second Cycle Timing	What to Assess	Green Light Criteria
Minimum: 12 weeks off	Testosterone recovery	> 80% of pre-cycle baseline
After blood work	Lipid profile	LDL < 130, HDL > 40
After goal review	Physique and strength	Significant further progress potential
After education	Next compound knowledge	Full understanding of any new compound